Interview Findings Mapped to TOGAF Phases A–E

Barangay San Antonio Health Department · UA&P Solution Architecture · Compiled October 20, 2026 from 11 interviews

1. Interview Log

#	Interviewee	Position	Main phases
1	Hon. Thomas Raymond U. Lising	Punong Barangay	A, B, E
2	Ms. Rosalin Soropia Labayan	Barangay Treasurer	B (finance), D (costs), E
3	Dr. Glynnis Lynn U. Bonilla	Medical Officer IV / Rural Health Physician	A, B, C, D, E
4	Ms. Nita N. Hardman, RN	Rural Health Nurse (2 parts)	B, C, E
5	Ms. Mafe B. Abong, RM	Rural Health Midwife (Oct 2)	A, B, C, D
6	Dr. Joseph P. Magpali, DMD	Rural Health Dentist (2 parts)	B, C, D, E
7	Mr. Christian Jay G. Vasquez, RND	Nutrition Officer II / Barangay Nutrition Action Officer	A, B, C, E
8	Ms. Mary Anne M. Verches	Barangay Nutrition Scholar	B, C, E
9	Mr. Tito F. Reyes	IT Officer / Medical Encoder	C, D, E
10–12	Ms. Rowena A. Intalan, Ms. Rosemarie T. Labandero, Ms. Procelyn L. Tatoy	Pasig Health Aides (written)	B, C, E
13–15	Ms. Joan Marie E. Barcenas, Mr. Perfecto S. Pestaño Jr., Ms. Josefina G. Roa	Barangay Health Workers (written)	B, C, E
16	Mr. Jeson C. Medez	Security Officer (written)	B, C, D
—	Ms. Abong, Ms. Verches	Written follow-ups	B, C

Not interviewed (out of scope by team decision): OIC health aide Janneth D. Correa, utility (Ms. Vilma D. Lendio), Barangay IT team. Kagawad Rustia: profile compiled from others' accounts (Section 9).

2. Key Numbers

Area	Metric	Value	Source
Population	Barangay population	~13,000 (12,307 per midwife); ~17,000 incl. transients	Vasquez, Magpali, Abong
Population	Children 0–59 months	798–799	Verches, Vasquez
Population	Tanay relocatees who still use the center	~1,300	Magpali
Patients	Patients last month	657 (360 consultations + 297 medicine pickups)	Health center (verified Oct 22)
Patients	On maintenance medicine	200–300 per month	Hardman
Patients	Referrals out	~30 per month (~30%)	Hardman
Patients	Dental patients	10–15/day, Tue–Thu only	Magpali
Staffing	Dentist-to-population ratio	~1 : 13,000–17,000 (WHO: 1 : 5,000)	Magpali
Targets	Achievement vs. own caseload	80–90% (DOH population targets unrealistic)	Bonilla

Area	Metric	Value	Source
Targets	OPT Plus validity threshold	≥ 80% coverage	Vasquez
Budget	Barangay health & nutrition budget	~₱700,000 / year	Treasurer
Budget	Barangay-bought medicines (2026)	₱150,600 (₱150–200k typical)	Treasurer
Budget	Feeding program	₱614,000	Treasurer
Budget	Employee annual physical exam	₱280,000	Treasurer
Budget	Nutrition budget 2026 → 2027	₱450,301 → ₱650,809 (100% approved)	Vasquez
Budget	Feeding cost per child per day	₱70 (Nutribun alternative: ₱6)	Vasquez
Budget	Reimbursement of barangay medicine purchases	₱0	Kapitan, Treasurer
Budget	PhilHealth YAKAP medicine allowance	₱20,000 per member per year	Treasurer
Costs	Health center internet	PLDT, ~₱3,000 / month (barangay-wide ~₱17,000)	Health center (verified)
Costs	Staff paying for own cloud storage	₱200–400 / month (out of pocket)	Vasquez
IT	Internet outages	~5 per month	Bonilla
IT	Free Google Drive limit vs. OPT file size	15 GB vs. 2–3 GB per OPT file	Vasquez
IT	Work group chats (nurse alone)	30+	Hardman
Process	Medicine delivery after request	~1 week, often partial	Hardman
Process	2027 budget proposal deadline	End of October 2026	Kapitan

3. Phase A: Vision

Goals for 2027

Stakeholder	Goals
Kapitan Lising	(1) First 1,000 days infant health; (2) holistic senior care; (3) chronic disease (diabetes, hypertension, cholesterol). #1 to fund: medicines.
Dr. Bonilla	(1) Health promotion and prevention, esp. resistant condo residents; (2) quality and patient safety via a queuing system; (3) better health information management.
Mr. Vasquez	Aligned with the Philippine Plan of Action for Nutrition (PPAN) and SDG "No Hunger by 2030"; reduce stunting ~20%, wasting, obesity, micronutrient deficiency. 2027–2029 plan built on Social Behavior Change Communication.
Dr. Magpali	Oral health promotion and preventive dentistry; DOH "70-20" and "3-20" teeth goals; restart his "Ngiti San Antonio" program (stopped by the pandemic).

Business drivers

- Residents' top complaint: **medicine supply**; second: doctor availability (partly solved by a Rizal Medical Center online consultation tie-up).
- Hypertension cases rising faster than DOH/LGU supply; city buys in bulk for ~30 barangays through slow procurement.
- Condo residents resist government health services and often deny entry, even with signed letters.
- Paperwork load: duplicate encoding and hard-copy submissions take staff away from patient care.

Constraints (outside the center's control)

- City EMR is outsourced, off-the-shelf software; only the vendor can change it.
- Minor process or system changes need City Health Officer approval; major ones need the Mayor.
- Medicine and dental supplies come from City/DOH procurement and bidding.
- Staff salaries and most positions sit on the City payroll; dentist salary grade set nationally.
- Tanay, Rizal relocation site: no formal claim by Tanay; services, even water, come from Pasig.

Governance tension to reconcile: Kapitan says the barangay has a "free hand" and only informs the City out of courtesy, especially for barangay-funded work. Dr. Bonilla and Ms. Hardman say any system or process change needs City approval. Likely rule: barangay-funded operations are autonomous; City systems, City-issued equipment and clinical processes need CHD approval.

4. Phase B: Daily Work

Core processes

Process	Steps today	Pain points
Medical consultation	Registration → aides take vitals and measurements → Tito Reyes encodes → nurse assesses BP → doctor consults, encodes diagnosis and medicines → YAKAP prescription in a separate system → nurse explains and dispenses	Paper chart search for old patients; slow internet in the morning; doctor writes on paper AND types; no queuing
Dental visit	Appointment (walk-in for new, phone/text for old) in 15–60 minute slots → assistant pulls record, takes BP and temperature → dentist treats → assistant dispenses	Separate dental EMR (eMED) and PhilHealth system, so double encoding
Medicine logistics	Monthly request to CHD → delivered ~1 week later by CHD vehicle with requisition and delivery slips → stored in cabinets and barangay plastic bins → daily manual utilization log → monthly reconciliation → dispensed only on prescription	Partial deliveries; manual tracking; stock seen "visibly"; barangay buys the gap with no reimbursement
Referral	Nurse or doctor refers to Rizal Medical Center, Pasig City General Hospital or a hospital of the patient's choice; aides escort by barangay vehicle when needed	~30 per month; no laboratory on site
Program reporting	Daily logs → monthly, quarterly and annual reports per program (NTP/TB, NIP, mental health, smoking cessation, adolescent) to CHD via Google Drive; DOH inventory reports; printed hard copies carried to the old City Hall by aides on rotation	Redundant Google Drive forms ask for data already submitted; hard copies still required
Operation Timbang Plus	Year-round prep → January–March weighing at center, daycare, 5 Ortigas preschools, condos and Tanay → validation → city → region → national	Condo access; heat and distance; Tanay trips; must reach 80% coverage
Budget and procurement	Barangay Council prepares annual budget → Kapitan approves → City Council adopts. Urgent needs go to Kapitan or Kagawad; speed depends on amount. Projects: posting → bidding → voucher → check → liquidation	Treasurer keeps a hand-written ledger alongside the bookkeeper's computer

People and roles

- Nurse supervises aides and BHWs; BNS reports to the nutritionist, not the nurse.
- Pasig Health Aides (City) and BHWs (barangay) have the **same job description**.
- Employment types: most staff moved from barangay to City payroll (Kapitan); aides are job order with 6-month renewals (Bonilla); BNS went from honorarium to casual (6-month contracts) to permanent in ~2021.
- Cross-coverage: nurse and midwife swap immunization; midwife encodes when Tito is out.

- Part-time presence: nutritionist Tue–Thu; dentist Tue–Thu (covers Oranbo Mon and Fri plus a school).
- Tech literacy (Bonilla): clinical staff and encoder 5/5; aides ~2/5.

5. Phase C: Data and Applications

Application	Owner	Used for	Users
City EMR (eMed / "Pasig system")	LGU (outsourced vendor, ~2023)	Patient records, diagnosis, prescriptions, auto-generated reports (e.g., morbidity) sent to Medstat and FHSIS	Encoder, doctor, midwife, dentist (nurse does not use it)
Dental eMED	LGU	Dental records and PhilHealth claims	Dentist, dental assistant
PhilHealth HCI Portal / YAKAP	PhilHealth	Member registration, YAKAP prescriptions (own QR code)	Doctor, encoder
LMIS	LGU/DOH	Receiving medicines, vaccines, nutrition commodities	Encoder
ITIS	DOH	TB program	Nurse
e-OPT Plus tool	National Nutrition Council	OPT data entry	BNS, nutritionist
Google Drive / Sheets	Personal accounts (15 GB, no custom domain)	Nearly all program reports and nutrition data	Everyone
Messenger / Viber	Personal accounts	Team and program communication (30+ group chats)	Everyone
Barangay clearance system	Barangay	Fee collection	Treasurer's office

Data issues

- **Double or triple encoding:** paper chart + EMR; EMR + PhilHealth YAKAP (5–10 medicines retyped per patient; two separate QR codes); dental eMED + PhilHealth.
- EMR was built **without consulting the dentists**; dental diagnoses and common medicines were missing until a recent update.
- Medicine inventory is fully manual (beginning balance, daily utilization, monthly reconciliation).
- Work data lives on **personal** Google accounts and chats; staff pay for extra storage themselves.
- Access to the nurse's program records is granted by verbal consent; paper records are locked; mental health records restricted to nurse, midwife and doctor.

6. Phase D: Technology

- LGU-issued laptops for doctor, nurse, midwife, dentist and nutritionist; barangay laptop for BNS; the City audits City-issued equipment about every 6 months.
- Internet: the LGU connection is weak or unpaid, so staff use the barangay's own PLDT line (~₱3,000/month for the health center) or phone hotspots; ~5 outages a month; slow in the mornings.
- The center cannot install software; the City approves and runs seminars for new programs.
- Passwords are managed by the IT side; staff do not know them.
- Dental equipment (DOH-standard autoclave sterilizer) provided by the City, plus donations.

7. Phase E: Requested Changes and Candidate Building Blocks

Gap (who raised it)	ABB: what is needed	SBB: possible solution	Center controls it?
Medicine shortages (Kapitan, Treasurer, Abong)	Medicine supply resilience	Barangay buffer budget line + simple digital stock tracker with reorder alerts	Partly
EMR and YAKAP double encoding (Bonilla, Magpali)	Single prescription entry	Recommend EMR–PhilHealth integration to CHD and vendor	No (City/vendor)
No queuing (Bonilla)	Patient flow management	Number-based queuing system; UA&P capstone pilot	Yes, with CHO approval
Paperwork overload (Hardman)	Encoding capacity	Dedicated encoder for nurse programs; merge redundant Drive forms	Partly
Hard copies carried to City Hall (Hardman)	Digital report submission	Push CHD to accept signed digital copies	No
Personal storage and chats (Vasquez, Hardman)	Official collaboration workspace	Barangay-provided Google Workspace domain; Viber or work-only accounts	Yes (barangay-funded)
Unreliable internet (Bonilla, Reyes)	Reliable connectivity	Backup connection; offline paper fallback kept	Yes (barangay ISP)
Low tech literacy of aides (Bonilla, Verches)	Staff digital skills	UA&P computer workshops	Yes
Low oral and preventive health uptake (Magpali, Bonilla)	Health promotion	Behavior-based campaigns (e.g., mirror reminders), condo partnerships	Yes
Dental online booking (Magpali)	Appointment scheduling	QR/online booking feeding the EMR	Partly

Timing: 2027 budget proposals are due November 2026; avoid January–March (OPT) and month-end reporting. Kapitan and Dr. Bonilla (Friday afternoon) are willing to review the final plan.

8. Written Answers: Field and Support Staff

How the frontline team works

- **Buddy system:** each week one health aide is paired with one BHW, with set tasks: record retriever, vital signs, medicine dispensing (Tatoy, Labandero).
- **Mornings at the center:** vital signs, finding and filing patient charts, bringing patients to the encoder for EMR entry, dispensing (Intalan, Roa, Pestaño).
- **Afternoons in the field:** house-to-house dengue ovitrap/larvatrap placement, visits to pregnant women and seniors on maintenance, condom and pill distribution (Labandero, Intalan, Tatoy).
- **Maintenance medicine pickup:** Monday, Wednesday, Friday, 1–4 PM, one month's supply per visit; tracked by patient booklet, record and utilization sheet signed by the patient (Tatoy, Pestaño, Roa).
- **Outreach:** OPT Plus (height, weight, vitamin A, deworming for 6–59 months), MR-SIA scheduled per street and condo, and 3-hour "pop-up wellness" free services in condos (Roa, Intalan, Pestaño).
- **Referrals:** doctor consults first; emergencies are referred to hospital or endorsed to the barangay EMS; when no doctor is present, aides bring the patient's record to Oranbo or Kapitolyo (Roa: about 3–5 times a year; Intalan: "two").
- **Ms. Josefina Roa is the equipment and records-room custodian**, keeps the key, and submits a monthly equipment inventory to DOH, LGU and barangay. Only the main door is locked (Medez).

Reports from aides and BHWs

Report	Frequency	Sent to
Ovitrap / larvatrap results	Every 2 weeks	Dengue Task Force office
Family planning "unmet need"	Every 3rd week of the month	Health center / CHD
Accomplishment report with DTR	Before end of month	Supervisor
Medicine utilization	End of month	Nurse
Pregnancies delivered, FP acceptors, households visited	Monthly	Midwife and nurse

Logbooks and forms named

ITR and patient chart; NCD form; ADHEP and breast cancer forms; GeneXpert and sputum logbook; TB logbook; mental health logbook; Health YAKAP logbook and prescriptions; maintenance monthly monitoring (medicine, quantity, date, BP); employee attendance; equipment borrowers' logbook; visitors' and meeting logbooks.

Training

Family planning (new guidelines, September), communication skills for PHAs, Breastfeeding Support Group refresher, OSCE for PHAs, nutrition program management and nutrition in emergencies orientations, PHA General Assembly. Attendance is set by a City memo listing who may go. Two BHWs had no seminar this year. Ms. Abong estimates **20–25 trainings a year** for staff.

Written follow-ups from Ms. Abong

- **Hiring and promotion:** City Health Office / City Government of Pasig.
- **IPCR:** rates overall performance: quality and efficiency of work, timely reports, professionalism, respectful patient care.
- **Coverage:** Nurse Hardman or Dr. Bonilla take her pregnant and family planning patients; aides and BHWs assist.
- **Weekly volume:** about 60 consultations + 30 medicine pickups.
- **Data flow:** each visit is first written on the ITR and patient chart, then entered into the EMR. Her reports combine data from aides, the encoder, the nurse and EMR-generated figures.
- **Condos in the barangay:** about 63 (verified).

What they would change

Person	Request
Intalan, Roa	Own computers for health aides and BHWs
Pestaño	Computerize all patient records
Roa	Easier chart filing and new-patient form entry; a system to update and download reports instantly
Verches	Laptop for direct field encoding and a complete weighing kit
Labandero, Tatoy	Nothing; the buddy system and teamwork already work well
Medez	Help for whatever the center needs; care of equipment

9. Stakeholder Profile: Kagawad Rachel Marie S. Rustia-Olarte

Not interviewed directly. Built from what other interviewees said, plus public barangay posts (secondary, unverified).

Role (from interviews)

What was said	Source
Co-chair of the Committee on Health; urgent health center requests (medicines, supplies, computers) go to her or to the Kapitan, and speed depends on the amount	Kapitan Lising

What was said	Source
"In charge" of barangay medicine purchases for the health center during shortages; the health center coordinates purchases through her	Treasurer
Regarded as the head of the health department on the barangay side; staff consult her for barangay matters ("For Kagawad, it's Kagawad Rachel; for me, it's Kap")	Ms. Abong (Oct 2)
Sometimes joins OPT Plus field weighing with the Barangay Nutrition Committee	Ms. Verches
In a work group chat with the Kapitan and health center staff; no set rules on separating work and personal chats	Nurse Hardman
Barangay "pop-up wellness" free services in condos (3 hours) run by the center	Ms. Roa

Public record (secondary)

- Re-elected Barangay Kagawad in BSKE 2023, serving with Punong Barangay Thomas Raymond U. Lising.
- Leads health and nutrition initiatives: Community Wellness Pop-up (free services at condos and office buildings), OPT Plus in preschools, maternal support bags for mothers who complete prenatal and postpartum checkups.
- Signed the 2018 agreement with CARA Welfare Philippines and Cats of Ortigas for Trap-Neuter-Return and vaccination; supports pet events.
- Listed as Information Officer in the barangay's Incident Command System.

TOGAF placement

- **Stakeholder map:** high power, high interest (controls barangay health funding; hands-on in programs). Manage closely.
- **RACI:** Responsible for barangay-funded health purchases; Consulted on principles and Phase A; Accountable with the Kapitan for funding barangay-side work packages.
- **Phase E fit:** natural sponsor for medicine buffer stock, staff computers, Google Workspace, and condo outreach (Wellness Pop-up).

10. Contradictions and Open Items

Topic	Conflict	Suggested handling
Patient volume	Earlier estimates differed	Resolved: 657 last month (360 consultations + 297 pickups)
Referral frequency	Hardman: ~30/month to hospitals and labs · Roa: 3–5/year escorted to other centers	Different things: all referrals vs. escorted transfers
Consultation days	Nurse and encoder differed	Resolved: Mon/Tue/Fri 8–11 AM; maintenance MWF afternoons
Cross-barangay record access	Abong: other centers can see records · Bonilla: they cannot	Ask Tito Reyes
Governance	Kapitan: autonomy · Clinical staff: City approval needed	Split by who funds and owns the asset
Tanay relocation	Verches: after a 2008–09 fire · Vasquez: ~2019	Resolved: around 2018 (health center)
Barangay electricity	₱200,000 or ₱800,000/month	Resolved: ₱200,000/month
Nutritionist's name	Jay or Dave	Resolved: Christian Jay G. Vasquez